

TRANSITIONS It's not uncommon for some of our consumers to have trouble transitioning from one setting to another, from one task to another, and/or from one staff to another. Conduct issues ranging from anxiety to refusals to agitation to aggression to AWOL can result from this issue. In extreme forms people have refused to move for hours and some have shown extreme fight/flight reactions and have become a danger to themselves and others. One result of the failure to transition is that people may be unable to access services, support and experiences that they would enjoy and which could greatly benefit them. HillTop has been referred people where their transition issues are the principal reason for a day program with behavioral support. Once this issue is successfully addressed, the person often showed remarkable progress and enjoyed our program.

Who are the people with transition issues?

People too often are not considered when plans are made, when changes in those plans occur, and when routines are broken. Staff may not realize that the program participant has had bad previous experiences in certain settings. Staff may not adjust plans when program participants don't feel well. Too often people are just rushed, bullied, uninformed and denied the right to say "no!", "not right this minute", or even "what's in it for me!" Another big issue is that there is no plan or tolerance for when people fail to transition at first. Some people are predisposed to transition issues. They include folks with the following

diagnoses and/or conditions:

DIAGNOSIS/CONDITION

Autistic Spectrum Disorder

TRANSITION ISSUE

Strong preference for routine and difficulty handling novel and changing circumstances. They may also be sensitive to some environmental features (lights, sound, waiting in line, crowds, even smell)

Down syndrome Obsessive/Compulsive disorder Thought Disorders (particularly with paranoid issues) Mood Disorders (particularly depressive) Epilepsy (after big seizures)

Heavily Sedated People

Strong preference for familiar people and routines and difficulty handling novel and changing circumstances. They can refuse to move for hours (including dropping weight) They may not transition until their routines are completed, checked and possibly repeated A sense of not being safe, of not trusting others, of being overwhelmed by changes They may withdraw, refuse to participate, refuse to move They may be exhausted, disoriented and even agitated following big seizures They may fall asleep, stagger when walking and have no interest in programming

CAUSES AND TREATMENTS FOR DIFFICULTY WITH TRANSITIONS: General

Causes:

CAUSE TREATMENT

Previous bad experiences in Learn history to determine what worked and failed in the past. Avoid similar circumstances recreating past failures by being sensitive to preference and signs of intolerance Rigid expectations for Try to keep many things the same while changing only a few things at a sameness time. Allow for observation of others first, permit partial participation, provide escape options, quit while people are still successful, and provide heavy incentives. Plan for an option should there be a failure to transition Intolerance for the unfamiliar Make them familiar first with pictures, stories, allow them to see first, lower demands and expectations, leave early, allow refusal Sensitivity to crowds, lights, Avoid the worst of it; provide adaptations (head phones, sun glasses, noise etc.). A successful strategy for Know history, avoid power struggle, give up control, provide incentives, taking control, getting withhold big attention, wait it out, stimulus change, (have someone else attention, and/or extracting try to move them) recognize and plan for failure to transition. concessions

Transitions Fears of being harmed, lack Build trust, address the reality of the fears, and teach relaxation and of trust, acute anxiety desensitization A strong dislike of a program Limit the number and type of transitions in their program and build that moves from place to tolerance over time place and uses community settings

Morning Transitions:

CAUSE TREATMENT

Fatigue, illness Work with home staff to promote enough rest, adjust pickup time, and allow for partial participation. Insist on nonparticipation at times (consult Director) Being rushed, treated rudely, Try to calm things down, lower expectations; provide incentives, report or confrontationally abusive staff, build trust and friendships Poor relationships with home See above. Try to provide support to staff by working with them (not for staff and/or family. them). Lack of support for attending Build support for the program or report issues to the HillTop Director the program. and inform Regional Center caseworker. Competing contingencies. Try to make staying home less attractive Avoiding getting ready in the Schedule bathing at night, finish grooming in DP, provide cheap AM (bathing, grooming, breakfast. eating).

Stress in AM.

Setting to Setting Transitions:

Eliminate it as much as possible

CAUSE TREATMENT

They don't want to leave Plan ahead in AM, communicate plans, provide incentives, be patient, something they like, avoid settings where always happens, try stimulus change (someone else inviting them to leave). They don't know where they See above, provide enhanced schedule (words, pictures, symbols). are going next. Have them involved in planning and revisions. They don't like where they Make what follows the worst activity the best activity. are going next.

They are sensitive to a break Apologize, slow down transition, allow for partial participation, and in routine, heavily reinforce any tolerance. They refuse to enter the car. Try to calm things down by slowing the expectation for transition, build rapport first, and provide incentives (they can sit where they want, choose the radio station, etc.). Keep things very calm and encouraging

They refuse to exit the car:

when they try to move toward the car. Use child safety locks for safety in case they try to exit a moving car and don't travel far. Never place an agitated person in your car as they may damage it and HillTop will not pay for the damage. Call the office if they fail to transition to get support. Place the car out of the sun, open the windows and have everyone exit the car who is willing (take your keys). Move the group a short distance away from the person and wait. Do not look at or discuss the situation with the person in the car and give them a lot of attention when they do leave.

Staff to Staff transitions:

CAUSE TREATMENT

They are very attached to their Respect this and avoid needless changes, have them meet others former staff person. while with favorite staff who also treat them well, explain the need for change and be calm and patient. They never met the new staff The new staff must be extra nice, use reinforcement more and person. better, try to follow old routines to limit other changes, and give the relationship time to build. They were not prepared in See above advance for the change. They know and dislike the new Respect opinions, seek resolution of grievances staff and/or their consumers.